Product Name:		Territory:
Aminoven Infant 10%		**MY**
Type of Packaging:	Dosage:	
Leaflet	**250 ml**	
Material number:	2-D-Matrix Code:	
M087929/02 MY	**M087929/02 MY**	
Pharma-Code (Laetus):	EAN-Code:	
-	**-**	
Dimension:	Font:	Size:
180 x 294 mm	**Arial**	**10**

Colours:	1. Draft	**18.06.2020, 09:00**
• **Schwarz**	1. Corr	**25.06.2020, 12:30**
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Operator:		
Roberto Grill		

Aminoven infant 10%

Solution for infusion

Composition
1000 ml contains:

L-leucine	13.00 g
L-isoleucine	8.00 g
L-lysine acetate = L-lysine 8.51 g	12.00 g
L-methionine	3.12 g
L-phenylalanine	3.75 g
L-threonine	4.40 g
L-tryptophan	2.01 g
L-valine	9.00 g
Arginine	7.50 g
L-histidine	4.76 g
Glycine	4.15 g
L-taurine	0.40 g
L-serine	7.67 g
L-alanine	9.30 g
L- proline	9.71 g
N -acetyl- L-tyrosi ne = L-tyrosine 4.20 g	5.176 g
N -acetyl- L-cysteine = L-cysteine 0.52 g	0.70 g
L-rnalic acid	2.62 g
Total amino acids:	100 g/l
Total nitrogen content	14.9 g/l
Titration acidity:	27-40 mmol/ NaOH/l
pH -value:	5.5 -6.0
Theoretical osmolarity:	885 mosm/l
Other constituents: Water for injections	

Indications
10% amino acid solution for partial parenteral nutrition, of infants (preterm and term newborns, babies) and young children. Together with corresponding amounts of electrolytes, carbohydrates, vitamins and fat emulsions as energy donors, the solution may serve for total parenteral nutrition.

Posology and method of administration
Aminoven infant 10% should be administered as constant intravenous infusion via central veins.
Maximum infusion rate:
up to 0.1 g amino acids/kg body weight and hour = 1.0 ml 1kg body weight/ hour

Maximum daily dosage:

• Ist year of living:	1.5 -2.5g amino adds/kg bodyweight
• 2nd to 5th year of living:	1.5 g amino adds / kg body weight
• 6th to 10th year of living:	1.0 g amino adds 1kg body weight
• IIth to l4th year of living:	1.0 g amino adds / kg body weight

The solution is administered as long as parenteral nutrition is required.

When use in neonates and children below 2 years, the solution (in bags and administration sets) should be protected from light exposure until administration is completed (see Section Warnings and Precautions for Use).

Remark:
Too rapid infusion may result in renal losses causing amino acid imbalance. Electrolytes should be administered according to the requirements.

Product Description
Aminoven Infant 10% is a clear, colourless to light yellowish 10% amino acid solution for partial parenteral nutrition in premature infants, babies and small children.

Pharmacological properties

i. Pharmacodynamics properties
The amino acids contained in Aminoven infant 10% are all naturally occuring physiological compounds. As with the amino acids derived from the ingestion and assimilation of food proteins, parenterally administered amino acids enter the body pool of free amino acids and all subsequent metabolic pathways.Amino acids are the building blocks for protein synthesis.

ii. Pharmacokinetic properties
The bioavailability of Aminoven infant 10% is 100%.
The amino acids in Aminoven infant 10% enter the plasma pool of corresponding free amino acids. From intravasal space, amino acids distribute to the interstitial fluid and, individually regulated for each single amino acid, into the intracellular space of different tissues as required.
Plasma and intracellular free amino acid concentrations are endogenously regulated within narrow ranges, depending on the agee, nutritional status and pathological condition of the patient. Balanced amino acid solutions such as Aminoven infant 10% do not significantly alter the physiologic amino acid pool when infused at a constant and slow speed.
Characteristic changes in the physiologic amino acid pool of the plasma are only foreseeable when the regulative function of essential organs like liver or kidneys are seriously impaired. In such cases special formulated amino acid solutions may be recommended for restoring of the homeostasis.
Only a small proportion of the infused amino acids is eliminated by the kidneys, depending on the maturity of the infant´s kidneys and general disease condition.
The biological half-lifes of amino acids in plasma depend on the age and metabolic situation of the pediatric patient.

Contraindications
As for all amino acids solutions Aminoven Infant 10% should be administered in the following conditions:-Disturbances in amino acid metabolism, acidosis, hyperhydration, hypokalemia. Patients with insufficient renal or hepaticfunction require an individual dosage. Attention in case of hyponatremia.

Remark:
When administering amino acids as part of the parenteral nutrition of prematuse infants, babies and small children thefollowing laboratory parameters must be regularly checked: urea-N acid-base balance, serum ionogram, liver enzymes, lipid level (where fat emulsions are used), water balance and if necessary serum amino acids levels.

Side effects
None known when correctly administered.

Interactions with other medications
Because of the increased risk of microbiological contamination and incompatibilities, amino add solutions should not be mixed with other drugs.

Remark:
The addition of drugs may alter the chemical and physical properties of the solution and therefore give rise to toxic reactions. Should it nevertheiess become necessary to add drugs to Aminoven infant 10%, it is imperative to ensure sterility, complete mixing and compatibility.
The same appliesfor the adding of lipid emulsions, trace elements and vitamins for complete parenteral nutrition mixtures.

Overdose (symptoms, emergency procedure, antidotes)
As with other amino acid solutions shivering, vomiting, nausea and increased renal amino acid losses can occur when Aminoven infant 10 % is overdosed or the infusion rate is exceeded. Infusion should be stopped immediately in this case. It may be possible to continue with a reduced dosage. In the event of hyperkalemia the infusion of 200 to 500 ml 10% glucose solution with an added 1 to 3 U modified insulin / 3-5 g glucose is advisable.

Special warnings and precautions for use
Frequent evaluation and determination of the following laboratory values should be recommended for monitoring parenteral nutrition in infants: urea-nitrogen, ammonia, electrolytes, glucose and triglycerides (when a fat emulsion is administered), acid-base and fluid balance, liver enzymes and serum osmolality.

Infusion via peripheral veins in general can cause irritation of the vein intima and thrombophlebitis. To minimize the risk of vein irritation, daily controls of the punction site are recommended.

Aminoven infant 10% is applicable as part of total parenteral nutrition regimen in combination with adequate amounts of every donors (carbohydrate solutions, fat emulsions), electrolytes, vitamins and trace elements.

Light exposure of solutions for intravenous parenteral nutrition, especially after admixture with trace elements and/or vitamins, may have adverse effects on clinical outcome in neonates, due to generation of peroxides and other degradation products. When used in neonates and children below 2 years, Aminoven Infant 10% should be protected from ambient light until administration is completed (see section Posoloty and Method of Administration).

Use during pregnancy and lactation
Studies with this product have not been carried out on pregnant women. However, the clinical experience with similar parenteral amino acid solutions has not shown evidence of risk in pregnant and breastfeeding women.

Shelf-life of the medicinal product as packaged for sale Aminoven infant 10% is assigned a shelf-life of 24 months.

Shelf-life after reconstitution according to directions
The addition of other components to the amino acid solution **Aminoven infant 10%** prior to application should take place under hygienic conditions ensuring that the solution is well dispensed. Unless other data are available, mixtures should be used within 24 hours.

Storage conditions
Store protected from light. Do not store above 30°C

Precautions
Do not use Aminoven infant 10% after expiry date. Use only clear solutions and undamaged containers. Keep drugs out of the reach of children.

Presentation
Available in 100 ml and 250ml glass bottles packed in cartons: 10 X 100 ml
10 X 250 ml

Date of Revision: July 2020

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KABI

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